

[DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity — monthly report

Project: [DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity
Reporting period: 2026-03-01 !' 2026-03-31
Prepared with DonorDesk

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Executive Summary

This document is made possible by the generous support of the American people through the United States Agency for International Development (USAID). The contents are the responsibility of GEC and do not necessarily reflect the views of USAID or the United States Government.

In its first month, the USAID-funded Turkana Maternal, Newborn and Child Health (MNCH) Strengthening Activity, implemented by GEC with the Lodwar Community Health Alliance (LOCHA) across 14 facilities in Turkana County, trained its first health worker cohort and began community-level delivery. March 2026 produced 8 activity records covering training, services, community outreach and data quality.

March was a start-up month, and most results sat below their monthly share of targets. The project trained 14 health workers in Emergency Obstetric and Newborn Care (EmONC), Integrated Management of Childhood Illness (IMCI) and essential newborn care (8 female, 6 male) against a life-of-activity target of 120. It completed 8 mentorship visits against a target of 84, and trained and kitted 40 community health volunteers against a target of 220. Service delivery began: 380 first antenatal care visits (target 4200), 160 skilled deliveries (target 2100) and 290 children under one receiving DTP3/Penta3 (target 3300). Some 66% of facilities submitted Kenya Health Information System (KHIS) monthly reports on time. Four or more antenatal care visits stood at 29% against a 45% target, below expectation, up from the 28 baseline. No equipment kits were delivered; the needs assessment and first tender were completed.

Recruitment of nurse mentors and the first procurement round took longer than planned, so services were only partly covered in early weeks. The project compressed the training calendar to recover the missed days. In April it will continue training cohorts, deliver the next equipment batch, and expand outreach and community activities.

Progress Against the Work Plan

March was a start-up month, and most results sat below their monthly share of targets across the Intermediate Results (IRs) because training, equipment and community structures were still being put in place.

IR1: Health worker capacity and facility systems.

The project trained the first cohort of 14 health workers in Emergency Obstetric and Newborn Care (EmONC), Integrated Management of Childhood Illness (IMCI) and essential newborn care at Kanamkemer Health Centre (8 female, 6 male) and completed 8 facility clinical mentorship visits across the 14 supported facilities in Turkana County, with mentors deployed from the second week of March. No facilities received the essential Maternal, Newborn and Child Health (MNCH) equipment kit: the project completed the needs assessment for all 14 facilities and issued the first tender, but United States Agency for International Development (USAID) procurement rules requiring three quotations delayed delivery.

IR2: Service delivery.

At the 14 supported facilities in Turkana Central, Turkana South and Loima sub-counties, the project recorded 380 first antenatal care visits, 160 skilled deliveries and 10 women referred for emergency obstetric care through the transport voucher scheme. Immunization services, including 18 outreach sessions, delivered DTP3/Penta3 to 290 children under one year (145 girls, 145 boys). The project delivered these services with county health staff and facility teams; start-up delays left services only partly covered in the first weeks.

IR3: Community engagement and data systems.

The project trained and kitted 40 community health volunteers (26 women, 14 men), reached 1800 people with community health education (1116 women and girls, 684 men and boys), screened 900 children aged 6-59 months for acute malnutrition with mid-upper arm circumference (MUAC) tapes (459 girls, 441 boys) and referred 60 children with acute malnutrition to treatment. The project held a monthly data review with 28 facility in-charges and records officers; 66 % of supported facilities submitted their Kenya Health Information System (KHIS) monthly reports on time.

Indicator performance table

Indicator	Month result	Cumulative result	Life-of-activity target	Percent of target
ANC4+ coverage (HL-OC1a)	29 %	29 %	45 %	64.4 %
Skilled birth attendant coverage (HL-OC1b)	42 %	42 %	60 %	70 %
DTP3/Penta3 coverage under 1 year (HL-OC1c)	63 %	63 %	80 %	78.8 %
Caregivers naming three danger signs (HL-OC2a)	0 %	0 %	60 %	0 %
Health workers trained in EmONC, IMCI or essential newborn care (HL-1.1a)	14	14	120	11.7 %
Facility clinical mentorship visits (HL-1.1b)	8	8	84	9.5 %
Facilities equipped with essential MNCH kit (HL-1.2a)	0	0	14	0 %
First ANC visits (HL-1.3a)	380	380	4,200	9 %
Skilled deliveries (HL-1.3b)	160	160	2,100	7.6 %
Emergency obstetric care transport voucher referrals (HL-1.3c)	10	10	180	5.6 %
Children under 1 year receiving DTP3/Penta3 (HL-1.4a)	290	290	3,300	8.8 %
Outreach immunization sessions (HL-1.4b)	18	18	180	10 %
Facilities submitting KHIS reports on time (HL-1.5a)	66 %	66 %	90 %	73.3 %
CHVs trained and kitted (HL-2.1a)	40	40	220	18.2 %
People reached with community health education (HL-2.2a)	1,800	1,800	18,000	10 %
Children 6-59 months screened with MUAC (HL-2.3a)	900	900	9,000	10 %
Children with acute malnutrition referred to treatment (HL-2.3b)	60	60	650	9.2 %

The caregiver danger-signs figure (0 % against a 60 % target) is based on partial records.

Behind-plan indicators and corrective action.

Most indicators are below their monthly share of the target because March was a start-up month: training, equipment and community structures were still being put in place. Mentor recruitment and the first procurement round took longer than planned, so services were only partly covered in the first weeks and no facilities received equipment kits. The project deployed mentors in the second week instead of the first and compressed the training calendar so missed days are recovered in April.

Planned activities for the next reporting period.

The project will continue training cohorts of health workers and hold monthly mentorship visits, recover the missed training days in April under the compressed calendar, deliver the next batch of equipment kits to the remaining facilities, continue outreach antenatal clinics and voucher reimbursements to transporters, continue outreach immunization sessions with monthly cold-chain checks, train the next CHV cohort and supervise those already trained, continue dialogue days and radio messages, continue monthly MUAC screening and track referral completion, and follow up on late-reporting facilities through the monthly KHIS data review.

Gender Equality and Women's Empowerment

Sex-disaggregated results were recorded for the March people-level indicators and activities. The project trained 14 health workers (8 female, 6 male) in EmONC, IMCI and essential newborn care. It trained and kitted 40 CHVs (26 female, 14 male). Community health education reached 1800 people (1116 female, 684 male). Immunization reached 290 children under one year (145 female, 145 male). MUAC screening covered

